# Recommended, reimbursed — and still not completed.

A partnership brief on closing the gap between the preventive care that is recommended and the preventive care that actually gets finished — in the safety-net settings where the gap is widest.

## THE PITCH IN 30 SECONDS

PROPOSED · IN DEVELOPMENT

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| PROBLEM | U.S. colorectal-cancer screening sits at **63.5% against a 72.8% target** — and 44.89% in Federally Qualified Health Centers (CY2025 UDS; Michigan 48.74%). The services work; **completion** is what fails. |
| SOLUTION | A proposed **weekly routine**, run by the site's own staff: find who is overdue, rank by three transparent rules, remove friction, and follow every positive test to its next step, tracked on a weekly dashboard. A predictive model is on the roadmap — not in the first pilot. |
| EVIDENCE | Every lever is already recommended by the Community Preventive Services Task Force (mailed FIT +16.1pp, reminders +15.3pp, navigation +13.6pp). Honest caveat: real-world gains depend on **how well each step is carried out** — STOP CRC averaged ~3.4pp — which this design exists to protect. |
| COST | The host clinic contracts the implementation from month one: a feasibility rate for months 1–3, the full rate from month four only if it decides to continue, both agreed in writing before the start. The evaluation is funded separately, through the academic partner. Published benchmark: **~$90** per additional person screened by mailed FIT (Pignone et al., JGIM 2021; total cost, 2019 dollars; $54.83 direct per person screened) — not the pilot's cost. |
| THE ASK | One clinic to run a **12-month pilot**: a **3-month feasibility phase**, then the full pilot to month 12 if it works. Step one is a 30-minute conversation. |

## THE PROBLEM — ADOPTION, NOT SCIENCE

**~8%**
of U.S. adults 35+ receive all 15 high-priority preventive services (Borsky, *Health Affairs* 2018).

**63.5% → 72.8%**
CRC screening vs. the Healthy People 2030 target — barely off baseline (2023).

**44.89%**
CRC screening in Federally Qualified Health Centers, CY2025 UDS (Michigan 48.74%) — the gap concentrates here.

## WHICH ONE ARE YOU? — DIFFERENT PARTNERS, DIFFERENT VALUE, SAME ENGINE

HEALTH CENTERS / FQHCS

### Improve a measure you already report

Host the pilot — raise a UDS/HEDIS measure you already report, inside your existing staffing. You contract the implementation from month one and decide at month three whether to go on.

FUNDERS / PARTNERS

### Fund a cost-effective, scalable model

Fund either side: a grant to the host clinic for the implementation, or to the academic partner for the evaluation; months 4–12 released only if the feasibility phase shows the routine can run. Criteria agreed before results; a path from one site to a network.

RESEARCHERS

### Co-design a publishable pilot

An independent evaluation: before and after against the site's own trend, with a comparison clinic where possible, reported with RE-AIM. Plan published first; authorship by contribution. Principal investigator: to be confirmed.

## THE PILOT LOOP — THREE CORE PILLARS, AND A THREE-PILLAR ROADMAP

CORE · WHAT THE FIRST SITE RUNS

- Service redesign & friction reduction
- Interoperable data infrastructure — one nightly coded 19-column file for the pilot, dates as day counts, key kept at the site
- Implementation enablement — training so the site runs and sustains it

ROADMAP · AFTER A VALIDATED PILOT, NOT IN THE FIRST PILOT

- Predictive AI, explainable
- Agentic & conversational outreach
- Algorithmic equity & governance

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Identify → Rank by transparent rules (days overdue, days since a positive test, prior no-shows) → Reduce friction → Communicate (literacy-tailored) → Escalate (community-health-worker outreach) → Measure, incl. diagnostic follow-up after a positive result. No patient messaging is automated in the first pilot.
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## THE DEMONSTRATION

### A proposed 12-month colorectal-cancer completion pilot, with a stop at month three.

One Southeast Michigan health center below the UDS benchmark, with outreach staff and a quality-improvement lead. **Months 1–3: feasibility** — kits sent and returned, positive tests navigated and booked, days to a booked colonoscopy, staff hours, data completeness. **Month 3: decision** — the site continues to month 12 if the feasibility measures are met, or it stops. **Months 4–12: full pilot.** The site keeps its workflow and its data; HSREP is implementation lead, contracted by the site (twenty hours a week in months 1–3, thirty from month four); an academic partner, still to be confirmed, evaluates independently, before and after against the site's own trend with a comparison clinic where possible and a target published in advance. First readout about month six; results published whichever way they go.

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An HSREP initiative in development. All designs are illustrative and future-tense; no patient data has been collected. The first pilot runs on transparent rules and existing staff; AI components are a roadmap item. Operational materials will be reviewed by